

醫健通

eHealth

香港特別行政區政府 HKSARGOVT

Code of Practice for Using Electronic Health Record for Healthcare

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1. INTRODUCTION

1.1. PRACTICAL GUIDELINES FOR USE OF eHEALTH

This Code of Practice (COP) is issued by the Commissioner for the Electronic Health Record (“the Commissioner” or “eHRC”) under section 52 of the Electronic Health System Ordinance (Cap. 625) (“eHealth Ordinance”).

This COP serves as an important guideline for Healthcare Providers (HCPs) and Healthcare Professionals (HCProfs) to comply with the requirements of the eHealth Ordinance. It sets out the major principles, standards and best practices for using eHealth in a secure and proper manner and it should not be regarded as exhaustive. While some flexibility in approach is allowed, it is strongly recommended to adhere to the practices outlined herein to safeguard the security and privacy of Health Data for all participants. Failure to implement appropriate measures may elevate the risk of security or privacy incidents and could ultimately jeopardize the continued access to eHealth.

Mere non-compliance with this COP by itself does not render the person to any criminal proceeding unless such action of breach in itself constitutes an offence under the eHealth Ordinance or other ordinances.

This COP issued by the eHRC is different from documents / codes with similar titles issued by other regulatory bodies for governing the conduct and behaviour of HCProfs or by the Office of the Privacy Commissioner for Personal Data (Privacy Commissioner) for overall protection of personal data under the Personal Data (Privacy) Ordinance (PD(P)O) (Cap. 486).

1.2. TARGET AUDIENCE

Section 2 - *COP for Management Executives, Administrative and Technical Staff using eHealth* provides general and practical guidance for Management Executives, Administrative and Technical Staff working in HCPs who have participated in eHealth.

Section 3 - *COP for Healthcare Professionals using eHealth* provides general and practical guidance specific for HCPs using eHealth for sharing health information for providing healthcare to Healthcare Recipients (HCRs).

1.3. USE OF COP

Reading this COP facilitates understanding and compliance with the eHealth Ordinance and other relevant ordinances to safeguard HCRs' privacy and confidentiality for using eHealth. In compiling this COP, reference has been made to similar guidelines issued by various authorities and renowned organisations overseas where electronic medical or patient record systems have been implemented (see Annex 4.1).

Participants are recommended to read this COP in conjunction with the eHealth Ordinance, PD(P)O and other references quoted throughout this document. Other useful references include guidelines, notices, newsletters and relevant updated information issued by the Electronic Health Record (eHR) Office (see Annex 4.8). Participants are also reminded to refer to the Codes of Practice and Guidelines issued by the Office of the Privacy Commissioner regarding protection of personal data privacy (see Annex 4.10).

The eHRC may, from time to time, revise the whole or any part of this COP and publish further guidelines and other requirements for operation of eHealth. Latest version of the document can be accessed at the website of eHealth (www.ehealth.gov.hk).

1.4. DEFINITION OF TERMS

Authorized Person (AP)

means a person who is authorized by the HCR to submit forms in relation to registration and consent matters on behalf of the HCR under the requirements of the eHealth Ordinance

Commissioner for the Electronic Health Record (eHRC)

means the public officer appointed under section 48 of eHealth Ordinance (Cap. 625) to operate and maintain eHealth

Data Access Request (DAR)

means a request under section 18 of PD(P)O

Data Correction Request (DCR)

means a request under section 22(1) of PD(P)O

Data Sharing

means the act of providing or obtaining any sharable data of a Registered HCR through eHealth

Electronic Health System (eHealth or System)

means the information infrastructure for keeping the eHRs of Registered HCRs, sharing and using of data and information contained in those eHRs, and for providing support in connection with, or facilitating, the provision of healthcare to Registered HCRs or health management by Registered HCRs

Electronic Health System Ordinance (Cap. 625) (eHealth Ordinance)

means the ordinance which provides for the establishment of eHealth, the sharing and using of data and information, and the protection of eHealth, data and information; and to provide for incidental and related matters, which is amended by the Electronic Health Record Sharing System (Amendment) Ordinance 2025 with effect from 1 December 2025

Electronic Health Record (eHR)

means, in respect of any Registered HCR, the record of data and information of the Registered HCR (or any part of it) kept in eHealth; and, in respect of any HCR who was once registered, but is no longer registered, the record of data and information in relation to such HCR that was kept in eHealth

Electronic Health Record Office (eHR Office)

means the administrative office set up under the eHRC or engaged by the eHRC for operation and administration of eHealth

Health Data

means the data and information relating to the health condition of, or to the healthcare provided or to be provided to, the HCR; or (if applicable) any life-sustaining treatment that is not to be provided to the HCR as stated in an instruction

in an advance medical directive (as defined by section 2(1) of the Advance Decision on Life-sustaining Treatment Ordinance) of the HCR

Healthcare Professional (HCProf)

means, in relation to this COP, a Hong Kong healthcare professional as specified in Section 4.4

Healthcare Provider (HCP)

means a person that provides healthcare in Hong Kong or elsewhere

Healthcare Recipient (HCR)

means an individual for whom healthcare has been performed, is performed, or is likely to be performed in Hong Kong or elsewhere

Index Data

means the personal particulars of the HCR that identify the HCR for the operation of eHealth

Identifiable

means the identity of the HCR is ascertainable from the data or information

Joining Consent (being given before 1 December 2025)

Joining Consent being given by an HCR or a SDM (if applicable) before 1 December 2025, i.e. the effective date of the amendments to the eHealth Ordinance pursuant to the Electronic Health Record Sharing System (Amendment) Ordinance 2025, is for the eHRC to: (i) share data with a Prescribed HCP which has obtained a Sharing Consent from the HCR; and (ii) (if the Prescribed HCP has made a healthcare referral to another Prescribed HCP) share with that other Prescribed HCP the HCR's sharable data relevant to the healthcare referral.

Non-identifiable

means identity of an HCR is unascertainable from the data or information

PD(P)O

means the Personal Data (Privacy) Ordinance (Cap. 486)

Personal Data

means any data, according to PD(P)O:

- a. relating directly or indirectly to a living individual;
- b. from which it is practicable for the identity of the individual to be directly or indirectly ascertained; and
- c. in a form in which access to or processing of the data is practicable

Prescribed Healthcare Provider (Prescribed HCP)

means:

- a. the Department of Health;
- b. the Hospital Authority;

- c. the Primary Healthcare Commission;
- d. a healthcare facility managed or controlled by the Government, by the Hospital Authority, or by an HA subsidiary; or
- e. a registered HCP

Privacy Commissioner

means the Privacy Commissioner for Personal Data established under section 5(1) of the PD(P)O

Registered Healthcare Recipient (Registered HCR)

means an HCR who is registered under section 8(1) of eHealth Ordinance

Relevant Healthcare Provider (Relevant HCP)

means:

- a. a Prescribed HCP; or
- b. a recognized non-Hong Kong HCP

Relevant Joining Consent

means a joining consent given by a Registered HCR or a SDM (if applicable) on or after 1 December 2025, i.e. the effective date of the amendments to the eHealth Ordinance pursuant to the Electronic Health Record Sharing System (Amendment) Ordinance 2025, for the eHRC to: (i) obtain from a Relevant HCP or a recognized non-Hong Kong public health record system any Sharable Data of the Registered HCR; (ii) obtain from a Specified HCP any specified Health Data of the Registered HCR; (iii) provide a Relevant HCP or a recognized non-Hong Kong public health record system, to which the Registered HCR has given a Sharing Consent, any Sharable Data of the Registered HCR; and (iv) (if a Prescribed HCP has made a healthcare referral to another Prescribed HCP) provide to that other Prescribed HCP the Registered HCR's Sharable Data relevant to the healthcare referral.

Relevant Sharing Consent

in relation to a Specified HCP, means a Sharing Consent given before 1 December 2025.

Sharable Data

means the Index Data and the Health Data of an HCR and any other data or information of the HCR that is, in the eHRC's opinion, necessary for the proper functioning of the eHealth.

Sharing Consent

means a consent given by a Registered HCR or a SDM on behalf of an HCR (if applicable) to a Relevant HCP or a recognized non-Hong Kong public health record system under section 12 of eHealth Ordinance.

Specified Healthcare Provider (Specified HCP)

means an HCP who is required to provide specified data of an HCR to eHealth in the form and manner specified by the eHRC and within the specified period, pursuant to section 26Q of the eHealth Ordinance.

Substitute Decision Maker (SDM) (see Annex 4.2)

means an eligible person giving consent on behalf and in the name of an HCR under the requirements of eHealth Ordinance.

Use

in relation to data or information in an eHR, includes disclosure or transfer of the data or information.

2. CODE OF PRACTICE FOR MANAGEMENT EXECUTIVES, ADMINISTRATIVE AND TECHNICAL STAFF USING eHEALTH

2.1. REGISTRATION AS HEALTHCARE PROVIDERS IN eHEALTH AND PROMULGATION OF eHEALTH

- 2.1.1. HCPs meeting the registration requirements set out in the eHealth Ordinance may apply to the eHRC for registration in eHealth.
- 2.1.2. HCPs should maintain an updated registration record, including the business registration, contact persons, details of participating hospital(s) or clinic(s) and service locations, etc. They should inform the eHRC timely for changes in business nature and clinical services and provide all necessary supporting information for verification.
- 2.1.3. HCPs should withdraw from eHealth if they no longer fulfil the registration requirements, e.g. change of nature of services or termination of business.
- 2.1.4. HCPs should observe the conditions of registration of HCP in eHealth set by the eHRC (see Annex 4.8).
- 2.1.5. HCPs should be aware that their registration may be suspended or cancelled by the eHRC due to breaching of any requirement specified by the eHRC. Any such suspension or cancellation of registration may affect access and use of their HCRs' records in eHealth.
- 2.1.6. HCPs should follow recommendations given by the eHR Office for promulgation of eHealth.
- 2.1.7. HCPs should be aware that the eHR Office may from time to time update the information of Relevant HCPs¹ through various platforms (e.g. on internet:

¹ Information may include name, service location of HCPs and scope of data being shared to eHealth.

www.ehealth.gov.hk) for public awareness.

2.2. HANDLING REGISTRATION OF HEALTHCARE RECIPIENT

- 2.2.1. HCPs should observe the requirements set out by the eHRC for registering HCRs in eHealth (see Annex 4.8).
- 2.2.2. HCPs should ensure accurate capture and verification of HCRs' information during registration.
- 2.2.3. HCPs should submit a copy and retain appropriate original supporting documents of the HCRs for verification.
- 2.2.4. HCPs should observe the conditions (see Annex 4.2) which would allow a SDM to act for an HCR who is incapable of giving consent in registration and related matters.
- 2.2.5. HCPs should ensure that their administrative staff handle an HCR's and/or his/her SDM's Hong Kong Identity Card with care and in accordance with the guideline(s) issued by the PCPD when handling the HCR's registration and consent matters.
- 2.2.6. HCPs should take reasonable steps to ensure that an HCR and his/her SDM understand and agree with the purpose of using their personal data for:
 - (a) giving consent to join eHealth;
 - (b) giving Sharing Consent to HCP; and
 - (c) updating registration information (e.g. withdrawal from eHealth or revoking consent to an HCP) and related matters.
- 2.2.7. HCPs should update registration information of an HCR and his/her SDM if necessary and timely inform the eHR Office (e.g. HCR's name, sex, date of birth, number of Hong Kong Identity Card or other travel documents).

2.3. OBTAIN HEALTHCARE RECIPIENT'S CONSENT FOR eHR ACCESS

- 2.3.1. HCPs should obtain explicit, informed consent from an HCR or his/her SDM (if applicable) for:
 - (a) the HCR's registration in eHealth (including the giving of a Joining Consent); and
 - (b) sharing² of the HCR's Sharable Data through eHealth.

² If a Sharing Consent is given before the effective date of the amendments to the eHealth Ordinance (i.e. 1 December 2025), the former relevant provisions continue to apply in relation to the consent on or after the said effective date as if the former relevant provisions had not been amended by the Electronic Health Record Sharing System (Amendment) Ordinance 2025.

- 2.3.2. HCPs should take the following actions to ensure that the HCR's or his/her SDM's consent (if applicable) is valid and well-informed by:
- (a) providing sufficient, relevant and comprehensible information (e.g. Participant Information Notice, pamphlets, posters, etc.)
 - (b) obtaining consent from SDM of the HCR who is incapable of giving consent (see Annex 4.2); and
 - (c) confirming with the HCR or his/her SDM that the consent is voluntary.
- 2.3.3. HCPs should be aware of the general principles of handling consent given by HCR:
- (a) An HCR can give consent to register with or withdraw from eHealth, and give or revoke Sharing Consent, unless he/she is a minor under age 16 or if he/she is an adult and there is evidence that he/she is incapable of giving consent.
 - (b) For a minor or an HCR who is incapable of giving consent, consent should be given by his/her SDM.
- 2.3.4. HCPs should verify the identity of an HCR and his/her Authorized Person (if applicable) who submits forms on behalf of the HCR or SDM (if applicable) who handles registration and consent matters on behalf of the HCR.
- 2.3.5. HCPs should verify the identity of a SDM and confirm with the SDM that he/she has read and understood the Participant Information Notice, in particular the Important Notes for SDM Handling Registration Matters on Behalf of an HCR.
- 2.3.6. HCPs should be acquainted with the types of persons eligible to act as a SDM for a particular class of HCR as stipulated by the eHealth Ordinance (see Annex 4.2), who may give a substitute consent for that class of HCR to register with or withdraw from eHealth and to give Sharing Consent(s) to HCP(s).
- 2.3.7. HCPs should always request an HCR's own expressed preference where possible. If the HCR clearly expresses his/her intent, the HCP should carefully assess whether his/her case indeed requires any SDM.
- 2.3.8. HCPs should be aware that where there is no other eligible SDM available and the HCP considers that it is in the best interest for the HCR, the HCP can choose to give consent for registration (including Joining Consent) and sharing in eHealth, by appointing a designated person under its charge to assume the role and perform the tasks serving as the SDM of the HCR.
- 2.3.9. HCPs should be aware that Sharing Consent being given after the effective date of the amendments to the eHealth Ordinance, i.e. 1 December 2025, will be in effect until the HCR's registration is withdrawn from eHealth or cancelled by the eHRC or the consent is revoked. However, if a Sharing Consent is given before the said effective date, the former relevant provisions continue to apply

in relation to the consent on or after the effective date as if the former relevant provisions had not been amended, i.e. it can be an “Indefinite Sharing Consent” or a “One-year Sharing Consent”.

- 2.3.10. HCPs should be aware that Sharing Consent given by an HCR to the HCP is in effect until it is being revoked or the registration of the HCR is being withdrawn or cancelled.
- 2.3.11. HCPs should not share health information of HCRs who have withdrawn from eHealth or whose registration has been cancelled.
- 2.3.12. It is advisable that HCPs should inform an HCR, as far as practicable, before accessing his/her eHR and be aware that the HCR will receive a notification from eHealth of the access to his/her eHR in a form chosen by the HCR, including but not limited to the following:
 - (a) electronic message [e.g. Short Message Service (SMS) and notification in eHealth App];
 - (b) postal mail; and
 - (c) email.
- 2.3.13. HCPs should provide HCRs with access to their organisation’s privacy policy document(s) and information about the kinds of data from their health record system that will be shared and the purposes of sharing to eHealth.

2.4. HEALTHCARE PROVIDERS TO MANAGE ANY AUTHORIZED USER'S USER ACCOUNT

- 2.4.1. HCPs should be responsible for creating and maintaining user account(s) for any authorized user³ in eHealth, including checking and updating their HCPs' registration status for validation in a timely manner.
- 2.4.2. HCPs should seek agreement from the authorized user(s), if necessary, for the creation of user account(s) in eHealth and verification of the professional registration status of user(s) (if applicable) for access to eHealth.
- 2.4.3. HCPs should close the account of any departed person in eHealth in a timely manner after the last day of service of the person.
- 2.4.4. HCPs should issue appropriate authentication means (e.g. log-in passwords), according to the guidelines issued by the eHR Office to their HCP(s) to access eHealth (see Annex 4.8).
- 2.4.5. HCPs should ensure that only authorized HCPs with the "need-to-know" can access the eHR of HCRs under their care in accordance with the pre-defined professional role(s) and access rights.
- 2.4.6. HCPs should take reasonable and practical steps to ensure that their authorized user(s) respect(s) and has/have adequate awareness and knowledge of personal privacy, information confidentiality and system security⁴.
- 2.4.7. HCPs should ensure that their authorized user(s) is/are aware that using HCRs' information from eHealth for direct marketing is forbidden.
- 2.4.8. HCPs should take reasonable and practicable steps to ensure that the HCP(s) is/are properly observing the security control measures (e.g. using log-in passwords properly).
- 2.4.9. HCPs should appoint appropriate administrative and technical staff as contact person(s) to communicate with the eHR Office for matters relating to eHealth operation, including user account management (please see Annex 4.9 for Roles and Responsibilities of User Administrator in eHealth).
- 2.4.10. HCPs should supervise and monitor their staff in carrying out administrative and technical duties, including but not limited to:
 - (a) registering and managing registration information of HCP in eHealth;
 - (b) registering and managing registration information of HCRs in eHealth;
 - (c) registering and managing registration information of HCP in eHealth;
 - and

³ Authorized user may include any person authorized by respective HCP, who has clinical, administrative or technical duties and uses or supports operation of eHealth.

⁴ Reasonable and practical steps may include setting out confidentiality as an obligation under the terms of employment and human resource management policy and providing regular staff training and reminders and notice to staff.

- (d) performing regular reporting, exceptional reporting and cooperating with the eHR Office in audit on eHealth operations.

2.5. MANAGE HEALTHCARE PROVIDERS' OWN CLINICAL RECORDS

- 2.5.1. HCPs should maintain clear and updated clinical records for the HCRs. eHR should not be taken as a replacement of HCP's own health records.
- 2.5.2. HCPs should ensure the data in their medical record systems are accurate for sharing.
- 2.5.3. HCPs should provide the health information of an HCR after each episode of care as soon as possible if the information is readily and electronically available and within the sharable scope (see Annex 4.5), and the HCR has given a Relevant Joining Consent or a Relevant Sharing Consent to this HCP which was given before 1 December 2025 and remains in effect.
- 2.5.4. HCPs should be aware that any data obtained from eHealth shall become part of the HCP's health records and HCPs should define and follow their own data retention policy in accordance with the PD(P)O.
- 2.5.5. HCPs should ensure proper filing and record keeping, according to their own record management policies, of any HCR's health records which are obtained from eHealth, and prevent unauthorized access of these health records.

2.6. PROVISION OF SPECIFIED HEALTH DATA TO eHEALTH BY SPECIFIED HCPs⁵

- 2.6.1. The eHRC may require Specified HCPs who provide healthcare services in Hong Kong and possess specified Health Data of Registered HCRs to deposit the specified Health Data into eHealth. This requirement applies when the Registered HCR has given either a Relevant Sharing Consent to the Specified HCP or a Relevant Joining Consent, except where the Registered HCR has explicitly issued a notice prohibiting data sharing to eHealth.
- 2.6.2. Specified HCPs must ensure the establishment and maintenance of appropriate technical systems and controls that enable the secure, accurate, and timely provision of specified Health Data to eHealth in accordance with the technical

⁵After the Electronic Health Record Sharing System (Amendment) Ordinance 2025 takes effect on 1 December 2025, the Secretary for Health will be empowered to require specified HCPs to deposit specified important health data in the personal eHealth accounts of Registered HCRs. The specified health data, specified HCPs, and the specified period for providing the health data will be implemented later through subsidiary legislation.

and operational requirements prescribed by the eHRC.

- 2.6.3. Specified HCPs should deposit the data in the prescribed form and manner determined by the eHRC within specified timeframes, thereby ensuring that timely and accurate data is available in the HCRs' eHealth accounts.
- 2.6.4. Specified HCPs must fully respect and adhere to any explicit refusal notices from Registered HCRs and must refrain from depositing any specified Health Data without valid consent (i.e. a Relevant Sharing Consent or a Relevant Joining Consent).
- 2.6.5. In cases where the eHRC determines that a Specified HCP has contravened the data provision requirement, the eHRC may issue a written enforcement notice specifying the nature of the contravention, the relevant Health Data, the affected Registered HCRs, the deadline and manner for compliance, and inform the Specified HCP of the right to appeal. The Specified HCP must either comply with the notice or submit an appeal within the designated period.
- 2.6.6. Specified HCPs are required to maintain comprehensive records of data provision activities, consents obtained or refusal notices received, in accordance with good governance practices and to facilitate any audit or investigation as mandated by the eHRC.

2.7. ENSURE GENERAL SYSTEM SECURITY

- 2.7.1. HCPs should implement and monitor proper use of security control measures in eHealth set out by the eHR Office from time to time, which shall include the following:
 - (a) Keep and access enabled computers (i.e. with appropriate certification software) only in secured physical locations (e.g. access within secured workplace, clinic or office) and avoid access to eHealth in public area such as internet cafe or public library;
 - (b) Keep and maintain security in wired and wireless network for computers connecting to eHealth;
 - (c) Keep computer system and software updated with latest security patches applied;
 - (d) Use only licensed / legal computer software and with latest security patches applied and avoid using peer-to-peer software (e.g. Foxy or Bit Torrent, etc.);
 - (e) Install appropriate anti-virus and anti-spyware software;
 - (f) Ensure authorized users log off eHealth and local Electronic Medical Record (eMR) systems after use;
 - (g) Enable automatic screen-lock or screen-saver with password protection

on computer workstation and set up reasonable idle time;

- (h) Ensure authorized users observe password policies (e.g. use of strong password with regular updates, avoid writing down or sharing of passwords; change eHealth assigned passwords immediately after successful login for the first time);
 - (i) Record and manage access rights assigned to all authorized users according to the roles in delivering healthcare to the HCPs;
 - (j) Assign individual account for each authorized user and ensure that he/she will properly use the account with security log-on measures (e.g. log-in passwords) and protect it against unauthorized use (e.g. sharing with others); and
 - (k) Provide security and privacy awareness trainings to users regularly, ensuring the proper use of eHealth in accordance with appropriate security and privacy requirements.
- 2.7.2. HCPs should cooperate with the eHR Office for audit or investigation which is in relation to the operation of eHealth, if necessary.
- 2.7.3. HCPs should maintain relevant system audit logs about access to eHealth through local eMR system(s), if applicable (see Annex 4.8).
- 2.7.4. HCPs should perform regular monitoring and audit on system behaviour for identification of abnormality, intrusion and potential system fault or user misbehaviour, if applicable.
- 2.7.5. HCPs should report, as soon as possible, to the eHR Office any suspected or confirmed security incidents, privacy incidents and suspected or confirmed security weakness relating to the access or use of eHealth.

2.8. MAKE DATA SHARING TO eHEALTH SECURED⁶

- 2.8.1. HCPs should endeavour to comply with standards, policies and requirements on security and data sharing issued by the eHRC (see Annex 4.8).
- 2.8.2. HCPs should perform self-assessment and test with the eHR Office for data readiness and interoperability before sharing information to eHealth (see Annex 4.8).
- 2.8.3. HCPs should perform system connection test with the eHR Office for data sharing according to security requirements and other specifications (see Annex 4.8).
- 2.8.4. HCPs should provide the amended and updated records in local eMR to eHealth, if any, of the relevant records were previously shared to eHealth.
- 2.8.5. HCPs should perform periodic Security Risk Assessment and Audit (SRAA) of their own eMR system(s), if applicable, or perform appropriate security assessment and fix any identified security loop holes according to the requirements specified by the eHR Office for system connection (see Annex 4.8). Any identified security risks or non-conformance with the security requirements should be rectified in a timely manner.

2.9. HANDLING DATA ACCESS REQUEST AND DATA CORRECTION REQUEST

- 2.9.1. HCPs should advise an HCR to approach the eHR Office for Data Access Request (DAR) for personal data being shared in eHealth.
- 2.9.2. HCPs should handle Data Correction Request (DCR) in accordance with the relevant provisions in PD(P)O and eHealth Ordinance.
- 2.9.3. HCPs should be aware that DCR for *demographic* data (e.g. name, number of identity documents, date of birth or sex) in eHealth can be handled by either the eHR Office or a Prescribed HCP.
- 2.9.4. HCPs should be aware that DCR for the HCR's *health data* in eHealth should be reviewed by the HCP who contributed and shared that related health information to eHealth, according to the established workflows for handling such requests by the eHR Office (see Annex 4.8).
- 2.9.5. HCPs should update and provide corrected health record(s) to eHealth, as soon as possible, once an error of the HCR's health record is noted and rectified.
- 2.9.6. HCPs should exercise careful judgement to handle the DCR and to inform the HCR and the eHR Office the result of such request and the reason of refusal, if

⁶ This section applies for sharing of data from local electronic medical record system to eHealth

the request is being refused.

3. CODE OF PRACTICE FOR HEALTHCARE PROFESSIONALS USING eHEALTH

3.1. USER ACCOUNT

- 3.1.1. HCPs would require a user account, which should be created by the HCP(s) they are working for, to access and use eHealth. To access eHealth via different HCPs, HCPs need to maintain separate user accounts under respective HCPs.
- 3.1.2. HCPs should provide updated professional registration information to the eHR Office for verification.

3.2. UPDATE RECORDS FOR eHR SHARING

- 3.2.1. HCPs should keep clear, accurate and updated clinical records of HCRs and share them to eHealth in a timely manner.
- 3.2.2. HCPs should comply with the PD(P)O while maintaining and sharing HCRs' health records to eHealth and make sure that the information is accurate and not excessive.
- 3.2.3. HCPs should exercise reasonable care in handling personal information relating to other individual(s) in an HCR's record(s). Personal information of individual(s) other than the HCR in the HCR's clinical record(s) should not be collected and shared to eHealth unless it is deemed necessary and consent from the concerned individual has been obtained. Personal information may include name, contact information, date of birth and identity document number of the other individual(s) (e.g. HCR's family member(s)).
- 3.2.4. HCPs should advise an HCR to approach the HCP who provided the corresponding health information to eHealth for data correction if they genuinely doubt the correctness or accuracy of the health information in the eHR. It is advisable to document such observation in the HCR's record(s).
- 3.2.5. HCPs have the responsibility to assist the respective HCP in dealing with DCR for alleged incorrect information in an HCR's clinical record(s) in a manner and within a time frame as specified under the PD(P)O and to provide an updated record to eHealth, if that information has previously been shared to eHealth.
- 3.2.6. HCPs should exercise careful judgment in accepting or refusing a DCR from an HCR. If they are not satisfied that the information to which the request relates is inaccurate, they should inform the HCR or the requester the decision and reasons of refusal in accordance with the PD(P)O.
- 3.2.7. HCPs should document the reasons for refusal for DCR in the HCR's records in accordance with the PD(P)O and established DCR procedures as defined by

the eHR Office.

3.3. POINTS TO NOTE BEFORE ACCESSING A HEALTHCARE RECIPIENT'S eHR

- 3.3.1. HCProfs should ensure the access to an HCR's eHR is authorized.
- 3.3.2. HCProfs have the responsibility to exercise judgement on clinical grounds to determine whether and how much information from the HCR's eHR should be accessed.
- 3.3.3. HCProfs should have the autonomy and professional judgment to interpret the information in eHealth.
- 3.3.4. HCProfs should only access eHR of the HCR with the latter's expressed and informed Sharing Consent and there is a need-to-know about the HCR's health conditions according to the roles of providing healthcare to the HCR.
- 3.3.5. HCProfs should be aware that the HCR will receive a notification from eHealth for any access to his/her eHR.
- 3.3.6. HCProfs should be aware that, for any HCR who is incapable of giving Sharing Consent to an HCP for eHR access, individual HCProf may make emergency access to the eHR of the HCR if that is of paramount importance for provision of emergency treatment to the HCR and subject to the relevant HCP's own internal rules, if applicable. HCProfs are advised to document such access in the HCR's record and the justification in eHealth and should be aware that such emergency access is subject to audit.

3.4. POINTS TO NOTE WHEN VIEWING AND USING eHR

- 3.4.1. HCProfs should interpret information from eHealth with care as it may not be updated and complete. They should judge the need to verify with other sources of information, and ideally, with the HCR, especially when in doubt or inconsistency is noted.
- 3.4.2. HCProfs should not regard eHR as a substitute for personal communication with the HCR and other healthcare professionals.
- 3.4.3. HCProfs should document relevant and important decisions and discussions, if applicable, with the HCR regarding the information from eHealth (including date / time of information being created and accessed, significant findings and conclusion after discussion with the HCR).
- 3.4.4. HCProfs should be aware that they have no obligation to copy all information from eHealth into their own HCR's records.
- 3.4.5. HCProfs should clearly indicate the source of information, date / time of the information being accessed when copying information from eHealth into the HCR's records kept by them.
- 3.4.6. In general, reports for third parties (e.g. insurance claims or health check report) should be based on the HCProf's own clinical records and /or assessment of the HCRs. HCProfs should exercise care in case they have to use information from eHealth for such reference.
- 3.4.7. HCProfs should exercise diligence of care in explaining any information accessed through eHealth to HCRs and not to use them for alleging challenges or criticism in whatever means to disparage or depreciate the professional skills, knowledge, services or qualification of other HCProfs and/or HCPs. HCProfs are however not prohibited from making fair and honest comments on a colleague if the professional conduct or competence may be called into question.

3.5. RESPECT CONFIDENTIALITY OF HEALTHCARE RECIPIENT'S INFORMATION

- 3.5.1. HCProfs should respect confidentiality of information obtained from eHealth.
- 3.5.2. HCProfs should be aware that each access to any HCR's eHR will be logged, monitored and audited.
- 3.5.3. HCProfs should ensure prior and express consent is obtained from the HCR before disclosure of any information obtained from eHealth to any third party.

3.6. ENQUIRIES AND ASSISTANCE

- 3.6.1. Staff designated by HCPs as contact points may approach the eHR Registration

Office for assistance via e-mail (ehr@ehealth.gov.hk) or telephone (3467 6230).

4. ANNEXES

4.1. REFERENCES FOR DEVELOPING THIS COP

1. Advisory Guidelines For The Healthcare Sector. Singapore. 2014.
2. Australian Medical Association. AMA Guide to Medical Practitioners on the use of the Personally Controlled Electronic Health Record System. Australia. 2012.
3. Australian Government Minister for Health and Ageing. Concept of Operations: Relating to the Introduction of a Personally Controlled Electronic Health Record System. Australia. 2011.
4. British Medical Association. Confidentiality and Disclosure of Health Information. Guidance from the BMA's Medical Ethics Department. UK. 1999.
5. British Medical Association and Royal College of General Practitioners. The Good Practice Guidelines for GP Electronic Patient Records. 2011
6. British Psychological Society. Guidelines on the Use of Electronic Health Record. UK. 2011
7. Canada Health Infoway Inc. White paper on Information Governance of the Interoperable Electronic Health Record (EHR). Canada. 2007
8. Canada Health Infoway Inc. Electronic Health Record (EHR) Privacy and Security Requirements – Reviewed with Jurisdictions and Providers. Canada. 2005
9. Code of conduct / practices of healthcare professionals and relevant registration ordinances in Hong Kong (See Annex 4.6) and Code of Practices issued by healthcare organisations in Hong Kong (See Annex 0)
10. Department of Health of Western Australia. Practice Code for the Use of Personal Health Information. Australia. 2009
11. Department of Health. Contracted Service Providers Participating in the eHealth Record System: Frequently Asked Questions. Australia. 2013
12. Department of Health. Participation Agreement. Personally Controlled Electronic Health Record System. Australia. 2013
13. General Medical Council. Good Medical Practice. UK. 2006.
14. General Medical Council. Confidentiality. UK. 2009.
15. Hong Kong Medical Association & Independent Commission Against Corruption. Integrity in Practice. A Practical Guide for Medical Practitioners on Corruption Prevention.
16. Information Commissioner's Office. Data Sharing Code of Practice. UK. 2011
17. Information and Privacy Commissioner. A Guide to Personal Health

- Information Protection Act. Ontario. 2004
18. Medical Board of Australia. Good Medical Practice: A Code of Conduct for Doctors in Australia. Australia. 2009.
 19. National Health Service. The Care Record Guarantee. Our Guarantee for NHS Care Records in England. 2009
 20. National Health Service. Confidentiality. NHS Code of Practice. UK. 2003.
 21. National Health Service. Information Security Management - NHS Code of Practice. UK. 2007
 22. Office of Privacy Commissioner. On the Record. A Practical Guide to Health Information Privacy. New Zealand. 2011.
 23. Personal Data Protection Commission. A History and Theory of Informed Consent. New York: Oxford University Press. 1986
 24. Principles of Biomedical Ethics. New York: Oxford University Press 1994.
 25. Policies, guidelines & procedures and other relevant information released by the eHR Office for participating in eHealth (See Annex 4.8)
 26. References from the Office of the Privacy Commissioner for Personal Data (PCPD) (See Annex 4.10)

4.2. SDM ARRANGEMENT FOR eHR SHARING

For an HCR under 16, or aged 16 or above and who is of any of the relevant descriptions⁷, an eligible SDM can act on the person's behalf at the person's best interest on the following relevant occasions: giving Joining Consent and Sharing Consent, renewal or revocation of a Sharing Consent and request for withdrawal of participation in eHealth.

Persons incapable of giving consent	Persons who may act as SDM
(a) A minor (below 16 years old)	1. Parents 2. Guardian⁸ 3. Persons appointed by court to manage the affairs of the HCR 4. Family members or person residing with the HCR 5. The Prescribed HCP that provides, or is about to provide, healthcare to the HCR at the relevant time.
(b) HCR who is 16 years old or above and is incapable of giving the person's own consent because of certain relevant descriptions ⁷	1. Guardian⁹ 2. Director of Social Welfare or any other person as guardian under the Mental Health Ordinance(Cap. 136)¹⁰ 3. Persons appointed by court to manage the affairs of the HCR 4. Family members or persons residing with the HCR 5. The Prescribed HCP that provides, or is about to

⁷ The relevant descriptions are:

- Mentally incapacitated as defined by section 2(1) of the Mental Health Ordinance (Cap. 136) s2(1);
- Incapable of managing his or her own affairs;
- Incapable of giving a Joining Consent at the relevant time as defined in the eHealth Ordinance; and
- Incapable of giving a Sharing Consent at the relevant time as defined in the eHealth Ordinance.

⁸ Appointed under or acting by virtue of the Guardianship of Minors Ordinance (Cap. 13) or appointed by court.

⁹ Appointed under the Mental Health Ordinance (Cap. 136).

¹⁰ Appointed under s44A(1)(i), 44B(2A) or 59T(1) or 44B(2B) or 59T(2) of the Mental Health Ordinance (Cap. 136).

	provide, healthcare to the HCR at the relevant time.
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4.3. HEALTHCARE PROVIDERS IN eHEALTH

HCP that provides healthcare at one service location in Hong Kong may apply to the eHRC to be registered as a healthcare provider for the System for the location.

A person provides healthcare at one service location if the person –

- a. is licensed under the Private Healthcare Facilities Ordinance (Cap. 633) in respect of one private healthcare facility;
- b. is registered under section 5(2) of the Medical Clinics Ordinance (Cap. 343) in respect of one clinic;
- c. carries on the business of dentistry under section 12 of the Dentists Registration Ordinance (Cap. 156) at one premises;
- d. holds a licence issued under section 8(2)(a) of the Residential Care Homes (Elderly Persons) Ordinance (Cap. 459) in respect of one residential home, and engages a healthcare professional to perform healthcare at that home;
- e. holds a licence issued under section 7(2)(a), or a certificate of exemption issued under section 11(2)(a), of the Residential Care Homes (Persons with Disabilities) Ordinance (Cap. 613) in respect of one residential home for persons with disabilities, and engages a healthcare professional to perform healthcare at that home; or
- f. is a specified entity that engages an HCProf to perform healthcare at one premises.

The eHRC may register a Government department as an HCP for the System if the eHRC is satisfied that the department provides an HCProf to perform healthcare for any HCR.

4.4. ACCESS TO eHEALTH BY HEALTHCARE PROFESSIONALS¹¹

Access to eHealth would be allowed for the HCProfs specified as follows:

1. Registered medical practitioner (Cap. 161);
2. Registered nurse or enrolled nurse (Cap. 164);
3. Registered midwife (Cap. 162);
4. Registered dentist (Cap. 156);
5. Registered pharmacist (Cap. 138);
6. Registered medical laboratory technologist (Cap. 359A);
7. Registered radiographer (Cap. 359H);
8. Enrolled dental hygienist (Cap. 156B);
9. Registered chiropractor (Cap. 428);
10. Registered occupational therapist (Cap. 359B);
11. Registered Part I optometrist (Cap. 359F);
12. Registered physiotherapist (Cap. 359J);
13. Registered and listed Chinese medicine practitioner (Cap. 549);
14. A person whose name is listed on an accredited register (as defined in Schedule 1 to the eHealth Ordinance), including:
 - (a) Speech therapist;
 - (b) Audiologist;
 - (c) Dietitian;
 - (d) Educational psychologist; and
 - (e) Clinical psychologist.
15. A person specified below who provides healthcare in a healthcare facility that is managed or controlled by the Government, the Hospital Authority or an HA subsidiary; or is engaged by a healthcare provider that participates in any health care programme authorized by the Government, including:
 - (a) an audiology technician;
 - (b) a chiropodist or podiatrist;
 - (c) a dental surgery assistant;
 - (d) a dental technician or dental technologist;
 - (e) a dental therapist;
 - (f) a dispenser;
 - (g) a mould laboratory technician;
 - (h) an orthotist or prosthetist;
 - (i) an orthoptist;

¹¹ eHealth Ordinance Cap. 625, Schedule 1

- (j) a scientific officer (medical);
- (k) a bioinformatician;
- (l) a genetic counsellor;
- (m) a Chinese medicine pharmacy practitioner;
- (n) a Chinese medicine pharmacy dispenser or technician;
- (o) a medical social worker;
- (p) a speech therapist;
- (q) an audiologist;
- (r) a dietitian;
- (s) an educational psychologist; and
- (t) a clinical psychologist.

Sharing by different HCProfs at different phases will be reviewed from time to time and announced by the eHR Office.

4.5. SHARABLE CLINICAL DATA IN eHEALTH SHARING

This section is extracted from the *eHR Content Standards Guidebook*.

HCPProf should be aware that the eHRC will review, decide and update the sharable scope from time to time.

eHR Content	Definition
1. Personal Identification and Demographic Data	All information that is required to accurately and uniquely identify a person, including: 1. eHR person identifier; 2. identity data; 3. demographic data; and 4. next-of-kin data
2. Encounters / Appointments	A list of booked appointments and attended healthcare encounters (contact between a person and the healthcare practitioner who will assess, evaluate and treat a person). An episode is composed of one or more encounter(s).
3. Healthcare Referrals	Information that is required when a healthcare practitioner transfers all or a portion of a person's care to another healthcare professional.
4. Clinical Note / Summary	The clinical note / summary contains information that summarise the following : 1. Reason originating the encounter / episode and the person's condition during the initial encounter; 2. Adverse drug reactions, allergies and clinical alerts found during the encounter / episode (this information should also be separately sent to the eHR at the appropriate section); 3. Major diagnostic findings during the course of the episode; 4. Problems identified; 5. Significant procedures performed and other related therapeutic treatment, e.g. medication; 6. The person's condition, therapeutic orders or treatment plan while preparing a periodic

eHR Content	Definition
	episode summary or upon termination of an episode; 7. Follow-up arrangement; and 8. Education to the person / family Remarks: The sharing of obstetric records by HCPs is available since March 2020.
5. Allergies and Adverse Drug Reactions	Information on the type of biological, physical or chemical agents that would result in / is proven to give rise to adverse health effects. Details of the adverse reactions, if occurred, should also be included.
6. Diagnosis	All active and inactive significant health and social problems. A problem can be a diagnosis, pathophysiological state, significant abnormal physical sign and examination finding, social problem, risk factor, allergy, reaction to drugs or foods, or health alert.
7. Procedures	Any significant procedures that are done for diagnosis, exploratory or treatment purposes.
8. Birth Records	The basic information about the eHR participant's birth, e.g. place of birth, birth weight, maturity. Part of the information relating to birth would fall under the other sharable scope, e.g. diagnosis, procedure, assessment.
9. Medication	This includes medication ordered and/or dispensed/administered during the healthcare process.
10. Immunisation Records	Vaccines administered to the person.
11. Laboratory Reports	Result of the laboratory tests which are sub-classified according to the nature of the test, namely anatomical pathology, biochemistry, haematology, microbiology, virology, and other laboratory test.
12. Other Investigation Reports	Other diagnostic test results could be of diverse range as discrete data element or a full report of the diagnostic test. Images, e.g. clinical photos, tracing, could also be included.

eHR Content	Definition
13. Radiology Reports	Radiology results would include radiology report and images. They are sub-classified according to modality, e.g. plain x-ray, fluoroscopy, ultrasound, computed tomography, magnetic resonance imaging, nuclear medicine, angiography and vascular interventional radiography, non-vascular interventional radiography, positive emission tomography and others.
14. Medical Certificate	A medical certificate is a formal statement about the health status or situation related to an individual.
15. Observation and Lifestyle	Observation refers to the monitoring of body basic functions, including vital signs and body measurement (e.g. blood pressure and blood sugar records) while lifestyle describes the way of life a person lives (e.g. smoking and drinking habit). The information provides the overall living and health conditions of an individual.

4.6. CODE OF CONDUCT / PRACTICES OF HEALTHCARE PROFESSIONALS AND RELEVANT REGISTRATION ORDINANCES

	Registered Healthcare Professionals	Documents	Issuing Authority	Cap	Remarks
1	Medical Practitioner	Code of Professional Conduct for the Guidance of Registered Medical Practitioners COP for Doctors (Private Hospital Association)	Medical Council of Hong Kong www.mchk.org.hk Private Hospital Association www.privatehospitals.org.hk	Cap 161	Revised in Jan 2016 Revised in May 2020
2	Nurses (registered or enrolled)	Code of Ethics and Professional Conduct for Nurses in Hong Kong	The Nursing Council of Hong Kong www.nchk.org.hk	Cap 164	Revised in Jan 2015
3	Dentist	Code of Professional Discipline for the Guidance of Dental	The Dental	Cap 156	Revised in

		Practitioners in Hong Kong	Council of Hong Kong www.dchk.org.hk		Dec 2019
4	Dental Hygienists	No information found in the website of the Dental Council of Hong Kong and there is no COP for the Dental Hygienists		Cap 156B	
5	Chiropractors	Code of Practice for Registered Chiropractors	Chiropractors Council of Hong Kong www.chiro-council.org.hk	Cap 428	Revised in Jan 2017
6	Midwife	Code of Professional Conduct and Practice for Midwives in Hong Kong	Midwives Council of Hong Kong www.mwchk.org.hk	Cap 162	Revised in Mar 2016
7	Pharmacist	Code of Professional Conduct for the Guidance of Registered Pharmacists in Hong Kong	Pharmacy and Poisons Board	Cap 138	2017 Version

			of Hong Kong www.ppbhk.org.hk		
8	Medical Laboratory Technologist	Code of Practice for Registered Medical Laboratory Technologists	Medical Laboratory Technologists Board of Hong Kong www.smp-council.org.hk/mlt/	Cap 359A	Revised in April 2012
9	Occupational Therapist	Code of Practice for Registered Occupational Therapists	The Occupational Therapists Board of Hong Kong www.smp-council.org.hk/ot/	Cap 359B	Revised in Jul 2017

10	Optometrist	Code of Practice for Registered Optometrists	Optometrists Board of Hong Kong www.smp-council.org.hk/op/	Cap 359F	Revised in Apr 2022
11	Radiographer	Code of Practice for Registered Radiographers	The Radiographers Board of Hong Kong www.smp-council.org.hk/rg/	Cap 359H	1998 Version
12	Physiotherapist	Code of Practice for Registered Physiotherapists	The Physiotherapists Board of Hong Kong www.smp-council.org.hk/pt/	Cap 359J	Revised in Jan 2014

13	Chinese Medicine Practitioner a) Registered b) Listed	Code of Professional Conduct for Registered Chinese Medicine Practitioners in Hong Kong Code of Conduct for Listed Chinese Medicine Practitioners	Chinese Medicine Council of Hong Kong www.cmchk.org.hk	Cap 549	Revised in Jul 2015 Revised in Jan 2018

4.7. COP OF HEALTHCARE ORGANISATIONS IN HONG KONG

	Documents	Issuing Authority	Cap	Remarks
Clinics registered under the Medical Clinics Ordinance under Medical Clinics Ordinance (Cap. 343)	Code of Practice for Clinics Registered Under The Medical Clinics Ordinance (Cap 343)	Department of Health www.dh.gov.hk	Cap 343	2010 Version
Private hospitals and day procedure centres registered under the Private Healthcare Facilities Ordinance (Cap. 633)	Code of Practice for Private Hospitals Code of Practice for Day Procedure Centres	Department of Health www.dh.gov.hk	Cap 633	2022 Version

4.8. POLICIES, GUIDELINES & PROCEDURES AND OTHER RELEVANT INFORMATION RELEASED BY eHR OFFICE FOR PARTICIPATING IN eHEALTH*

General Policies and Guidelines

1. Participant Information Notice
2. Conditions of Registration of Healthcare Providers (Private Hospitals) in eHealth/
Conditions of Registration of Healthcare Providers (other than Private Hospitals) in eHealth
3. eHealth Privacy Policy Statement
4. eHealth Personal Information Collection Statements
5. Important Note For Healthcare Providers Who Perform Healthcare Recipient Registration And / Or Obtaining Joining And Sharing Consent In Electronic Health Record Sharing System
6. Training materials for the operation of registration centres
7. Guidelines on Management of Healthcare Recipient Index
8. Guidelines on Management of Healthcare Recipient (HCR) Data by Healthcare Providers
9. eHealth Data Retention Policy
10. Policy and Guidelines for Handling Data Access Request (DAR) and Data Correction Request (DCR) in eHealth
11. Guidelines and procedures for using Hong Kong Identity (HKID) Card for eHR
12. Frequently Asked Questions for eHealth

eHR Data Standards

13. eHR Content Standards Guidebook.
14. Editorial Guide on Hong Kong Clinical Terminology Table – Overview
15. Guide on Implementation & Maintenance of the Hong Kong Clinical Terminology Table

eHR Security and System Connection Guidelines

16. IT Security Policies for eHealth
17. Security Assessment Checklist for Participating in the eHealth
18. Data Interoperability Guide
19. Communication Protocol (Data Interface) Specification
20. eHR Adaptor Interface Specification
21. Process Report and Exceptional Reporting Requirement

*The policies, guidelines & procedures and other relevant information released maybe

updated from time to time. Please visit www.ehealth.gov.hk or contact the eHR Office for the latest information.

4.9. ROLES AND RESPONSIBILITIES OF USER ADMINISTRATOR IN THE ELECTRONIC HEALTH SYSTEM (eHEALTH)

1. To manage user accounts and related administrative matters for HCPs to facilitate their participation and use of eHealth. Such activities may include creation of user accounts with appropriate functions; proper assignment of access rights; maintenance of updated user list, user profile and professional registration information; and timely closure of user account(s) when necessary, etc.;
2. To act as liaison person to communicate and cooperate with the eHRC on matters relating to the use of eHealth including:
 - a. To collect feedback from users and to disseminate updates of eHealth operational information to users (including provision of training on proper use of the eHealth as and when required);
 - b. To handle enquiries or complaints;
 - c. To handle DCRs in accordance with the PD(P)O;
 - d. To report to the eHRC any suspected or confirmed data, privacy or security incidents;
 - e. To provide assistance and information to the eHRC and other relevant parties/agencies for facilitating the investigation of data, privacy or security incidents; and
 - f. To carry out regular audit, investigation and related matters;
3. To perform other administrative duties for the HCP as required by the eHRC for the effective operation of eHealth.

4.10. REFERENCE FROM THE OFFICE OF THE PRIVACY COMMISSIONER FOR PERSONAL DATA (PCPD)¹²

Office of the Privacy Commissioner for Personal Data

(<http://www.pcpd.org.hk/index.html>)

Personal Data (Privacy) Ordinance (Cap 486)

(http://www.pcpd.org.hk/english/data_privacy_law/ordinance_at_a_Glance/ordinance.html)

Data Protection Principles

(http://www.pcpd.org.hk/english/data_privacy_law/6_data_protection_principles/principles.html)

Code of Practice and Guidelines

(http://www.pcpd.org.hk/english/data_privacy_law/code_of_practices/code.html)

Guidance Note

(http://www.pcpd.org.hk/english/resources_centre/publications/guidance/guidance.html)

Information Leaflets

(http://www.pcpd.org.hk/english/resources_centre/publications/information_leaflet/information_leaflet.html)

Proper Handling of Data Access Request and Charging of Data Access Request Fee by Data Users

(http://www.pcpd.org.hk/english/resources_centre/publications/files/DAR_e.pdf)

Guidance on the Proper Handling of Data Correction Request by Data Users

(http://www.pcpd.org.hk/english/resources_centre/publications/files/dcr_e.pdf)

Code of Practice on the Identity Card Number and other Personal Identifiers - Compliance Guide for Data Users (July 2016)

(https://www.pcpd.org.hk/english/data_privacy_law/code_of_practices/files/compliance_guide_e.pdf)

¹² The references from PCPD listed in this COP are not exhaustive. PCPD may issue more Code of Practice or Guideline Note or other material or update any existing documents from time to time. Readers of this COP are advised to visit the internet website of the Office of the Privacy Commissioner for Personal Data for the latest information (<http://www.pcpd.org.hk/index.html>).

4.11. COMMON TERMS ABOUT USES OF eHR IN eHEALTH

